

考前題庫出題格式說明

※請依照下列步驟將考題繕打於下一頁:

1. 請依下一頁**標準範例格式**繕打考題(**請依規定出題數**)，並請委員出完題數後**檔案另存新檔時，請於檔名註明委員姓名及題數**，並請 mail 至學會信箱 tastro@tastro.org.tw，以利考題統整及事後查核，謝謝！

註：**符合出題格式之委員**，將由學會於會後給付出題酬勞費予出題委員(100 元/題)，故請於出題後檔案務必註明出題者及題數，謝謝！

※題目請以**中文出題(限定)**，但專有名詞應用英文(免加註中文)。

※題目類型為**四選一**，禁出『**以上皆非或是以上皆是**』之選項。

※參考文獻為教科書，排除期刊，考 trial 需是教科書中或 Major treatment guidelines (NCCN, ASTRO, EORTC…) 寫的。

※AJCC 第八版

※題目及選項文字切勿太過冗長。

※出題時務必註明該題的**「出處」**。

2. 本格式已設定好樣式，故請依下一頁範例格式繕打，切勿更改其樣式。
3. 開始撰打考題→先於工作列上點選”**格式**”，下拉式選單中點選”**樣式與格式**”，該樣式的格式設定將會出現在螢幕的右方，請使用該格式來控制考題的格式。
 - a. 點選 Question: **可用來控制題號(題目)**
 - b. 點選 Choice: **可用來控制答案選項**，自動產生 A) B) C) D)，若續按 Enter 會繼續產生 E) F)…選項，需到樣式選項點選 Answer
 - c. 點選 Answer: **可用來控制答案格式(本項請再文字註明，出題的小組科別細項)**
 - d. 點選 QuizLevel: **難易度**
 - e. Ref: **參考文獻**
 - f. Comment: **註解(文字請限於 4 行內)**

4. 以上步驟若無法執行，請您用**複製的方式**，將下頁的範例題目複製後，貼上出題次數，然後將原內容文字刪除，保留格式，再將新的考題內容文字加入。

貼心提醒：

1.請各委員於 110.06.28(星期一)前將出題檔 MAIL 至本會專屬信箱

tastro@tastro.org.tw, 信件主旨請註明：110 年甄審考前出題 xxx 委員 xx 科目

2.附檔-保密協議書填寫完後亦請於 110.06.28 日前一併與考題 mail 回傳本會信箱,

謝謝！

「請各出題委員繕打考題時，一定要使用此格式，請勿自行變更該格式」謝謝！

「以下為範例出題格式，請各委員出題時，請以下列格式繕打，切勿變更其格式，謝謝！」

1. () 有關於膀胱癌(Bladder Cancer)之常見致癌因子，下列何者關聯性最少？

- A) Smoking
- B) Cyclophosphamide exposure
- C) Aristolochic acid exposure
- D) Pelvic RT history

(1 答案): D (GU-bladder)

(1 難易度): 易

(1 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(1 註解): Pxxx,

2. () 下列有關於膀胱癌(Bladder Cancer)之常見風險因子與流病敘述，何者為非？

- A) 已開發國家發生率多於發展中國家
- B) 台灣每年的新診斷膀胱癌個案約為 2200 位
- C) 好發族群有：年紀 (> 70) 、男性、多重共病
- D) 鱗狀細胞癌 (Squamous cell carcinoma) 常與寄生蟲感染相關

(2 答案): A (GU-bladder)

(2 難易度): 中

(2 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(2 註解): 發展中國家較多

3. () 下列有關於膀胱癌(Bladder Cancer)之敘述, 何者為誤?

- A) Transitional cell (or urothelial) carcinomas 為最常見, 約佔 90%
- B) 約 70-80% 為 non-muscle-invasive bladder cancer (NMIBC), 早期診斷者存活率佳但治療後仍有很高的復發機會
- C) 一般而言 Muscle-invasive bladder cancer (MIBC) 預後差, 經治療後 5 年存活率約為 45-50%
- D) Metastatic disease 的平均存活時間經化學治療後約為 2 年

(3 答案): **D (GU-bladder)**

(3 難易度): 中

(3 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.
N Engl J Med 2020; 383:1218-1230

(3 註解): Metastatic disease 的平均存活時間經化學治療後約為 1 年, 免疫治療 maintenance 後才可達 2 年。

4. () 下列有關膀胱癌(Bladder Cancer) AJCC 8th edition 分期敘述, 何者正確?

- A) T4a 指的是 Tumor invades pelvis or abdominal wall
- B) N3 指的是 common iliac lymph node(s) metastasis
- C) 淋巴結顆數不影響分期
- D) M1b 指的是 lymph node(s) metastasis beyond common iliac region

(4 答案): **B (GU-bladder)**

(4 難易度): 中

(4 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(4 註解): T4a = involving prostatic stroma, seminal vesicles, uterus, vagina
N1 = 1 node, N2 = >1 node

M1a = Distant metastasis limited to lymph nodes beyond the common iliacs

M1b = Non-lymph-node distant metastases

5. () 根據 AUA risk stratification of non – muscle-invasive bladder cancer (NMIBC), 何者為誤?

- A) 依據 T stage, 腫瘤數量, 腫瘤大小, histology grade, 是否切除後一年內復發, 是否為 BCG failure, lymphovascular invasion, prostatic urethral involvement 等分為 low risk, intermediate risk, high risk
- B) 符合 Low risk tumor 可考慮定期膀胱鏡追蹤即可
- C) Solitary low grade Ta tumor, multifocal 符合 high risk
- D) Solitary tumor, carcinoma in situ (CIS), ≤ 3 cm 符合 high risk

(5 答案): C (GU-bladder)

(5 難易度): 中

	EAU	AUA/SUO
Low risk	Primary, solitary, Ta, LG, <3 cm, no CIS	LG solitary Ta ≤ 3 cm PUNLMP
Intermediate risk	All tumors not defined in low or high-risk	Recurrence within 1-year LG Ta Solitary LG Ta >3 cm LG Ta, multifocal HG Ta ≤ 3 cm LG T1
High risk	Any of the following: T1, HG, CIS, multiple and recurrent and large >3 cm Ta G1G2 tumors (all must be met)	Any of the following: HG T1, any recurrent HG Ta, HG Ta >3 cm (or multifocal), CIS, BCG failure with HG disease, variant histology, LVI, HG prostatic urethral involvement

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(5 參考文獻):

Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(5 註解): Intermediate risk

6. () 下列有關 Non-Muscle-invasive bladder cancer (NMIBC) 膀胱癌之斬斷與治療, 何者為誤?

- A) AUA classification low/intermediate risk group 在 TURBT 切除乾淨之後可考慮 single dose intravesical chemotherapy 後追蹤
- B) AUA classification intermediate risk group 以上建議 intravesical therapy

- C) 若檢體出現少見之 malignant histology (ex. micropapillary, plasmacytoid, sarcomatoid),
建議行 cystectomy
- D) BCG 的副作用較 mitomycin 與 gemcitabine 少

(6 答案): **D (GU-bladder)**

(6 難易度): 中

(6 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(6 註解): In patients at intermediate or high risk of recurrence and intermediate risk of progression, BCG with 1-year maintenance is more effective than chemotherapy for prevention of recurrence; however, it has more side effects than chemotherapy. For this reason, both BCG with maintenance and intravesical chemotherapy remain options.

7. () 有關 Muscle-invasive bladder cancer (MIBC) 膀胱癌治療，手術與放射治療之間的比較，下列何者為誤？

- A) 不論採取手術或放射治療，仍有 50-60% 的病人會產生遠端轉移復發
- B) 近代放射治療的 complete response rate 在 MGH cohort (Giacalone et al. Eur Urol 2017) 內約有 70-80%
- C) 一般而言，手術有較高的 sexual dysfunction 而放射治療有較多 bowel symptoms
- D) 目前並無 head to head 研究比較 radical surgery 與 definitive chemoradiation，但由大型 retrospective cohort 與 meta-analysis data，接受手術與接受放射治療病人的 local control 與 overall survival 相近

(7 答案): **A (GU-bladder)**

(7 難易度): 中

(7 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(7 註解): Surgical cohort 20-30% distant metastasis per Table 68.5.
RT cohort: Giacalone et al.¹³³ conducted an analysis of 465 patients with cT2-T4a bladder cancer

treated at a single institution with maximal transurethral resection of bladder tumor (TURBT) followed by concurrent cisplatin-based chemotherapy and RT. Median follow-up was 4.8 years for all patients and 7.5 years for surviving patients. **Seventy-six percent** of patients had a CR to induction chemoradiation, 84% after a visibly complete TURBT achieved a CR versus 59% with an incomplete TURBT ($P < .001$). When evaluated in 4-year intervals, the CR rate has improved from 64% in years 1986 to 1990 **to 96% in years 2010 to 2012**. One hundred twenty-five patients (27%) underwent salvage RC, 55 for less than CR and 70 for superficial or invasive recurrences. For those patients who achieved a CR, the 10-year actuarial rates for noninvasive, invasive, pelvic, and distant failures were 32%, 16%, 14%, and 29%, respectively. Five-, 10-, 15-, and 20-year DSS rates were 66%, 59%, 56%, and 50% (T2 Z 75%, 66%, 61%, and 58%; T3eT4a Z 50%, 45%, 45%, 36%), respectively.

8. () 下列有關 Bladder preservation radiotherapy protocol, 何者為誤?

- A) 北美做法為 split course, 於 39-40 Gy (1.8- or 2-Gy per fraction) 時進 second cystoscopy, 確認為 T0 or non-invasive disease 時, 才進行 consolidation RT to 64-68 Gy
- B) 英國與澳洲做法為一段式 radiotherapy, 可用 conventional fractionation or hypofractionation
- C) 兩種作法下, 皆約有 60-70% 病人可長期維持 functional bladder
- D) 北美做法傾向不治療 pelvic lymphatics 而英國與澳洲做法通常會治療 small pelvis field

(8 答案): **D (GU-bladder)**

(8 難易度): 中

(8 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(8 註解): In North America, split schedules often used are 39 or 40 Gy in 1.8- or 2-Gy fractions with an interval cystoscopy; patients with responding disease proceed to a total dose of 64 to 68 Gy. Generally, these schedules achieve long-term survival comparable to surgical series. A significant risk of cystectomy remains, however, with 22% undergoing immediate cystectomy, 13% delayed cystectomy for local recurrence, and **65% retaining** a functioning bladder.¹⁹⁵ In some countries, particularly the United Kingdom and Australia, an alternative schema is used. As in the trimodality approach, patients will undergo maximal TURBT for diagnostic and staging purposes. They can then be treated either with primary surgery (as elsewhere) or alternatively with either neoadjuvant chemotherapy or primary radiation therapy. In the United Kingdom, the radiation therapy itself is given as a single definitive course, usually to the whole bladder, only with no attempt to treat the nodes (as discussed below). Typical dose schedules

would be 64 Gy in 32 fractions or hypofractionated schedules such as 55 Gy in 20 fractions. Interestingly, the long-term bladder preservation rates in older series seem similar to the rates for salvage cystectomy postradiotherapy occurring in approximately one-fourth of patients managed with radiation therapy alone at 10 years median follow-up,¹⁹⁷ with around **two-thirds** of surviving patients retaining their bladders. The authors' own more recent chemoradiotherapy series suggests a higher rate of bladder preservation with the use of synchronous chemotherapy combined with full-dose radical radiation therapy.

9. () 下列有關 BC 2001 trial (*N Engl J Med* 2012;366:1477-88.), 有關於 muscle-invasive bladder cancer (MIBC) 治療之敘述, 何者為誤?
- A) 收案條件為 T2-T4a N0 M0 disease
 - B) 為 2 x 2 factorial design, 同時比較 RT alone versus chemoradiotherapy 與不同 RT fractionation (55 Gy in 20 fractions versus 64 Gy in 32 fractions)
 - C) 允許 Neoadjuvant chemotherapy, 結果分析對 locoregional disease-free survival 並無影響
 - D) Synchronous chemotherapy with radiotherapy 使用 fluorouracil and mitomycin C

(9 答案): **B (GU-bladder)**

(9 難易度): 中等

(9 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.

(9 註解): 同時比較 RT alone versus chemoradiotherapy 與比較不同 RT field (Standard whole bladder vs. Reduced high dose modified field)

10. () 下列有關 BC 2001 trial (*N Engl J Med* 2012;366:1477-88.), 有關於 muscle-invasive bladder cancer (MIBC) 治療之敘述, 何者為誤?
- A) Chemoradiotherapy 組完成治療的比例 (compliance) 約為 60%
 - B) Chemoradiotherapy 組有顯著較低的 locoregional failure rate, 也有較低的 salvage cystectomy rate
 - C) 兩組的 Pelvic relapse rate 皆在 10% 以下

D) 兩組五年存活率 (48% CRT vs. 35% RT alone) 未達顯著差異

(10 答案): A (GU-bladder)

(10 難易度): 中

(10 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.
N Engl J Med 2012;366:1477-88

(10 註解): Overall, 173 of 182 patients (95.1%) in the chemoradiotherapy group and 170 of 178 patients (95.5%) in the radiotherapy group completed radiotherapy at the target dose, with 172 patients (94.5%) and 166 patients (93.3%). Pelvic node relapse in 9 patients (4.9%) in the chemoradiotherapy group and 12 (6.7%) in the radiotherapy group.

11. () 下列有關 BC 2001 trial (*N Engl J Med 2012;366:1477-88.*), 有關於 muscle-invasive bladder cancer (MIBC) 治療之敘述, 何者為誤?

- A) 大多數 disease failure pattern 為 locoregional recurrence, 與 RT high dose field 大小無關
- B) 兩組皆無使用 adjuvant chemotherapy
- C) RT field 不同並不影響 grade 3-4 late toxicities
- D) 兩組 pelvic recurrence rate 皆低係因 RT field 包含 pelvic lymphatics

(11 答案): D (GU-bladder)

(11 難易度): 中

(11 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.
N Engl J Med 2012;366:1477-88

(11 註解): 不包含 pelvic field

12. () 下列有關 BC 2001 trial (*N Engl J Med 2012;366:1477-88.*), disease failure pattern 之敘述, 何者正確?

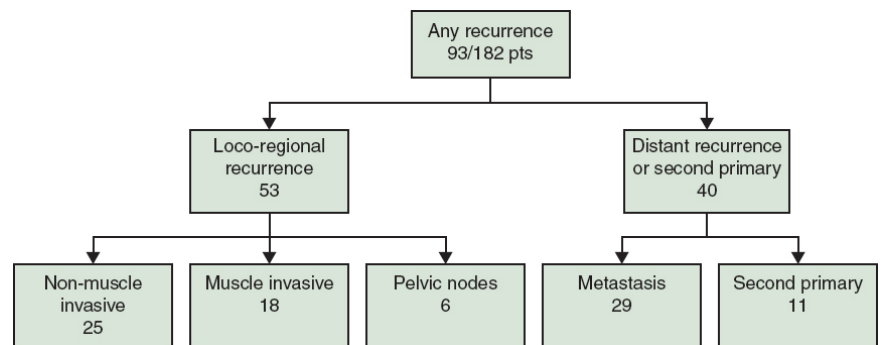
- A) 以 bladder local recurrence 為主

- B) 以 pelvic nodal recurrence 為主
- C) 以 distant metastasis 為主
- D) 以 secondary cancer 為主

(12 答案): A (GU-bladder)

(12 難易度): 中

(12 參考文獻): Principles and Practice of Radiation Oncology, Edward C. Halperin, David E. Wazer, Carlos A. Perez, Luther W. Brady; Chapter 68, p; 2019.



(12 註解): Fig. 68.8

13. () 下列較 ” 不 ” 合適作為 Bladder preservation 之病人? (1) T3-T4a disease (2) poorly functioning bladders (3) extensive CIS (4) multifocal disease (5) tumor-related hydronephrosis (6) N2-N3 disease

- A) (3) (4) (5)
- B) (3) (4) (5) (6)
- C) (2) (3) (4) (5) (6)
- D) (1) (2) (3) (4) (5) (6)

(13 答案): C (GU-bladder)

(13 難易度): 中等

(13 參考文獻): NCCN guideline, Bladder cancer, Version 3.2021 - April 22, 2021

(13 註解): Optimal candidates for bladder preservation with chemoradiotherapy include patients with tumors that present without moderate/severe hydronephrosis, are without concurrent extensive or

multifocal Tis, and are <6 cm. Ideally, tumors should allow a visually complete or maximally debulking TURBT. T2-T4aN0M0 is used as eligibility in BC2001 and RTOG trials. Table 68.6.